EL NIÑO ACTION BRIEFING · 2 OCTOBER 2026 · VERIFIED RE-RUN (REPLACES 28 SEPTEMBER)

Passo, Baguala, Ambon, Indonesia

El Niño reaches Passo as a long dry season, and the danger comes afterwards: water stored through the drought breeds dengue, and Maluku's dengue patients die at the highest rate in Indonesia. Get the health centre ready to treat dengue, and get the mayor to declare a drought emergency so water and money move before November.

IMPACT REGION	EXCESS HEAT DEATHS	WINDOW	DOMINANT HAZARD
Not retrievable at this resolution. Containing geography: Kota Ambon (357,289, mid-2024)	Indonesia 19,300 (10,400 in Dec–Feb). Kota Ambon ~24 by population scaling, our arithmetic	Study period Jun 2026 – Feb 2027	Drought and water shortfall (dry season Nov–Feb), then dengue and child diarrhoea

16 recommendations: 0 confirmed without correction, 10 corrected, 5 unverified, 1 contraindicated as sized. Three of the unverified items were carried from the 28 September run without being re-checked. **The funding target fell by more than a third against the 28 September report** ($88,700 → $55,500), because the village-fund placement was cut to what the channel can absorb. Exposure figure: Climate Impact Lab, *451,000 at Risk*, September 2026, pre-peer-review; heat only. **The Kota Ambon figure is our arithmetic, not an Impact Lab finding:** 19,300 × 357,289 ÷ 281,603,800 ≈ 24.5. No humid-equatorial Indonesian temperature–mortality study exists to check it against, and monthly mean temperatures here are only 25–27 °C.

* Suggested formulation: check with a health worker before sharing.

EXECUTIVE SUMMARY — AT A GLANCE



3.64°S 128.25°E, Indonesia · Natural Earth

- **The risk.** Water stored through the long dry season breeds dengue, and Maluku's dengue patients die at Indonesia's highest rate; after the 2015 El Niño, Ambon had 4 dengue deaths in January–April 2016, two in Passo.
- **First action.** Dinkes Kota Ambon and Puskesmas Passo ready the health centre to treat dengue by 1 November 2026, sustained to November 2027, with a written referral route to RSUD Haulussy.
- **The money.** $30,000 floor, $55,500 target, ~$107,000 ceiling; the top placement is $20,000 to Puskesmas Passo for filters or chlorine, covered containers, and ORS for under-5 households.
- **Do not.** Do not stand the response down when the rains return: the 2016 deaths came after the drought, and Ambon's normal June–November dengue season follows the rains.

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What changed since 28 September: BMKG now lists Kota Ambon at *Waspada* (alert) for early October; Ambon has still declared nothing; Maluku is still outside the IFRC drought operation; and new evidence moves dengue case management to the top.

WHAT THIS PLACE SHOULD DO — RANKED

Ranked by expected lives saved per unit of effort before and through the peak. Backing lenses are shown per action. Convergence was checked for shared bias. The RACI chart below assigns each one.

1. Get Puskesmas Passo ready to treat dengue from November 2026 through November 2027, with a written referral route to RSUD Haulussy

PARTIALLY CONFIRMED | PUBLIC HEALTH · HISTORIAN · EMERGENCY | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Dinkes Kota Ambon and Puskesmas Passo; Dinkes Provinsi Maluku for RSUD dr. M. Haulussy	Ready by 1 Nov 2026; sustained to Nov 2027	Training and IV-fluid buffer; not costed	Dengue cases becoming dengue deaths

Train staff in WHO warning-sign triage, keep a buffer of crystalloid IV fluid and haematocrit testing, and agree in writing who refers which patients to Haulussy and how. **Maluku reports the lowest dengue incidence in Indonesia but the highest case-fatality rate in most years since 2010** (Sasmono et al. 2026). Ambon's own record fits: in January–April 2016, after the last strong El Niño, the city had 37 cases and 4 deaths, two of them in Passo. Where clinicians were trained in dengue management, case fatality fell (Timor-Leste 12% to 3.6%; Bangladesh 1.81% to 0.95%).*

* Suggested formulation: check with a health worker before sharing.

Corrections: Ambon's usual dengue peak is June–November (Sasmono), so readiness can't be limited to January–April; the post-drought surge and the normal season both have to be covered. Maluku's dengue tracks the Indian Ocean Dipole (ρ 0.60) more than El Niño (Djaafara 2026). Haulussy is a provincial hospital that the province asked RS Fatmawati to help reorganise in July 2026, so the referral end is itself a capacity risk. The training evidence is before-and-after studies, not trials. Puskesmas Passo's beds and stock were not verified.

2. The mayor declares a drought emergency (Siaga Darurat Bencana Kekeringan) by about 20 October, on a BPBD assessment of supply failure

PARTIALLY CONFIRMED | EMERGENCY · ECONOMIST · HISTORIAN | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Wali Kota Bodewin Wattimena, on a BPBD Kota Ambon assessment	~20 Oct 2026	Free to sign; opens the Rp10 bn emergency reserve (BTT)	Tankers and money waiting on paperwork in December

The declaration is what opens the disaster route to the city's unexpected-expenditure reserve and lets BPBD request BNPB's on-call fund (Dana Siap Pakai). BMKG's early warning for the first ten days of October now lists Kota Ambon at *Waspada*; on 28 September it wasn't listed. Kota Tual declared Siaga Darurat by 6–8 September and is already distributing water. Ambon has declared nothing; the mayor's August statements promised water distribution with no status, budget or tanker count.

Corrections: Ambon is at the lowest tier in Maluku (eight areas are at *Awas*), so the case should rest on the water supply failing, not rainfall alone. The legal basis (UU 24/2007 and its implementing rules) and the Rp10 bn reserve figure could not be re-checked this run.

3. Map every Passo neighbourhood by water source, pre-contract tankers, and publish a fixed schedule by 31 October that serves uphill areas, Waimahu and Taholah first

PARTIALLY CONFIRMED | EMERGENCY · FRONTLINE · SKEPTIC · HISTORIAN | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
BPBD Kota Ambon with PDAM Tirta Yapono; RT heads and Pemerintah Negeri Passo for the map	Map by 20 Oct; schedule published by 31 Oct	Rp150–300k per 5,000 L load (unverified); ~Rp150–250m for Nov–Feb (lens estimate)	Water reaching whoever calls first, and households turning to unsafe sources

* Suggested formulation: check with a health worker before sharing.

Pemkot and BWS Maluku have named "dataran tinggi" (high ground) as most at risk, without naming neighbourhoods. Passo's Waimahu and Taholah only got public water access points in mid-2025 after waiting decades. Publish the schedule through RT heads, and test or chlorinate tanker water at the point of use rather than assuming it is safe.*

Corrections: the water points opened on 31 July 2025 per the village site ("26 years") or 2 August 2025 per Antara ("19 years"); both sources are cited. The utility is Perumda Air Minum Tirta Yapono, usually called PDAM Tirta Yapono. The 69% E. coli figure cited by a lens is from a different study (Irianto 2021); the Hansen 2026 result is 21.2% of 619 households above 100 CFU/100 mL, not Ambon-specific. Pemkot sent four tankers to Batu Merah in January 2026 and has one BNPB unit from December 2025, so fund operating costs, not trucks.

4. Give households with under-5s a filter or chlorine and a covered, tap-fitted container, and stock ORS through posyandu cadres

PARTIALLY CONFIRMED | PUBLIC HEALTH · PHILANTHROPIST · SKEPTIC | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Dinkes Kota Ambon; Puskesmas Passo and posyandu cadres	With the tanker schedule, before November	$20,000 placement (see funding)	Child diarrhoea from stored and trucked water

Filters roughly halve reported diarrhoea (RR 0.48, 95% CI 0.38–0.59; ceramic filters RR 0.39) and chlorination cuts it by about a quarter (RR 0.77) (Clasen et al. 2015, Cochrane). Where ORS was promoted, diarrhoea mortality fell 69% (Munos et al. 2010). Under-5 diarrhoea prevalence in Maluku was 13.38% (2017 survey), highest at age one, and stunting in Kota Ambon is 20.7%.*

* Suggested formulation: check with a health worker before sharing.

Corrections: expect roughly half the reported diarrhoea; any effect on deaths is uncertain. GiveWell's adjusted mortality estimate (14%) applies to chlorination only and is not statistically significant. The 69% is a pooled programme effect, not ORS alone. Zinc is kept as standard practice but no source was verified this run. Cochrane trials were mostly unblinded with self-reported outcomes. The Maluku prevalence figure is nine years old and province-wide.

5. A container programme that actually works: manufactured fitted lids or tap tanks, a scrub brush, and weekly checks by trained, supervised larva monitors, through August 2027

PARTIALLY CONFIRMED | SKEPTIC · HISTORIAN · PUBLIC HEALTH · FRONTLINE · EMERGENCY | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Posyandu cadres and jumantik (larva monitors) with Puskesmas Passo; Pemerintah Negeri Passo co-funds	**Start in October; sustain through August 2027**	**$17,500 placement plus village fund (see funding)**	**Drought-stored water becoming the next dengue outbreak**

Every container a drought response adds is a breeding site until a fitted lid and a weekly scrub prove otherwise. Severe drought raised dengue risk 1.85-fold across Southeast Asia, with a second effect about two months later as storage habits change (Ith et al. 2025). Community mobilisation without chemicals cut dengue infection 29.5% in the Camino Verde trial (Nicaragua and Mexico), and temephos added nothing there. In Yangon, improvised lids alone left 14.8% of containers positive in the dry season; lids plus weekly cleaning brought it to zero.

* Suggested formulation: check with a health worker before sharing.

Corrections: larvicide (temephos, "abate") only as an add-on, after a WHO bioassay on Ambon larvae. Resistance is documented across Indonesia (Surabaya, Padang, Pekanbaru and others), and Maluku has never been tested. Monitors need training and supervision: in Malang only 48.3% had good overall knowledge and 52.2% checked regularly. All trial evidence is from Latin America and Myanmar.

6. PMI Kota Ambon sends verified needs data up to PMI Pusat so IFRC can add Maluku to its Indonesia drought operation (MDRID030)

PARTIALLY CONFIRMED | ECONOMIST · EMERGENCY | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
PMI Kota Ambon via PMI Maluku and PMI Pusat	By 15 Oct 2026; the operation runs to 31 Mar 2027	Staff time	Ambon being left out of the only pre-arranged drought money

The operation (US$120,284, 200,000 people, 25 September 2026 – 31 March 2027) covers 20 provinces in three priority tiers; the word "Maluku" doesn't appear in its 26 pages. Its own trigger report says low rainfall is expected across "Sulawesi, Maluku and parts of Papua", and the document says coverage changes go through an Operations Update. That is the opening.

Realistic money is small: about US$0.60 per person if spread evenly, which is our division, not a costing. PMI must document water needs against the operation's targeting criteria first.

7. One message in Melayu Ambon, agreed by the Raja and Saniri, through the GPM congregation and, via the pela, Batumerah's imam; and a named cadre for every isolated elder

UNVERIFIED (EFFECT) | FRONTLINE | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Raja Negeri Passo and Saniri; GPM congregation; posyandu cadres; Batumerah's mosque leadership via the pela	October 2026	Volunteer time	Warnings reaching only one community

* Suggested formulation: check with a health worker before sharing.

Message: "tutup tampayang, kuras tiap minggu" (cover the water jar, scrub it weekly), where to report a water outage, the warning signs of dengue, and when to bring a child in. Passo (adat name Paukala Mandalise) holds a pela, a roughly 500-year alliance, with Batu Merah, a mainly Muslim negeri; the three villages renewed it at a joint *makan patita* on 18 May 2024 with the mayor present. Carrying one message through both is how it reaches both communities without looking like it came from one.*

Facts confirmed; the effect is not. No source shows pela or adat channels used for health messaging in Ambon (COVID-19 work there was door to door), so treat this as a reasonable hypothesis. Passo's own population is unsourced.

8. Scale up the city's health-insurance subsidy so lapsed members are active before the dengue season

PARTIALLY CONFIRMED — ALREADY UNDER WAY | ECONOMIST · PUBLIC HEALTH | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Pemkot Ambon with BPJS Kesehatan Cabang Ambon and Dinkes	Enrolment before Jan 2027	~Rp6.0 bn/year for ~12,000 more members (our arithmetic)	Families delaying hospital care because their card is inactive

98.25% of residents are enrolled in national health insurance (JKN), but only 76.78% are active (Sekda, 24 September 2026), below the 80% threshold. Pemkot has already put Rp8.9 bn of its 2026 budget into premiums for 16,021 people. Use of JKN in Maluku is low: 14.79% of members against 17.66% nationally (2021).

The link to fewer deaths is inferred, not measured. The Rp42,000 monthly premium could not be checked against the regulation.

Carried from 28 September, not re-checked this run

UNVERIFIED THIS RUN

Enforce the mayor's 15 August burning appeal through February, not just September, with Satpol PP and the Damkar post at Passo, and give the fire service its own water fill point. **Protect Passo's springs and its roughly 320 m mangrove belt**, test the conductivity of any well before use, and drill no emergency bore wells. **Count deaths weekly, pooled across Kota Ambon**, through Dinkes and the civil registry, because Passo alone is too small to show a change.

* Suggested formulation: check with a health worker before sharing.

WHO DOES WHAT

RACI CHART

R = Responsible (does the work) · **A** = Accountable (one per action; signs off and answers for it) · **C** = Consulted (asked before) · **I** = Informed (told after). Built from the owners named in the verified actions. No one here has been contacted or has agreed to a role; every row is a proposal.

Action	Wali Kota / Pemkot	BPBD Kota Ambon	Dinkes Kota Ambon	Puskesmas Passo	RSUD Haulussy / Dinkes Prov.	PDAM Tirta Yapono	Negeri Passo (Raja & Saniri)	RT heads	Posyandu cadres & jumantik	GPM & Batumerah mosque (pela)	PMI (Kota / Pusat)	IFRC	BPJS Kesehatan	BMKG	Damkar / Satpol PP
1. Dengue case-management readiness	I		A	R	R		I		I				C		
2. Drought emergency declaration	A	R	C			C	I	I			I			C	
3. Water map, tankers, schedule	I	A	C	C		R	R	R	I	I	C				
4. Water treatment, containers and ORS for under-5 households			A	R			C	I	R						
5. Fitted lids, weekly checks, larvicide only after bioassay		I	A	R			R	I	R	I					
6. Add Maluku to IFRC drought operation	I	C									R	A		C	
7. One message via adat, church, mosque; elder cadres			I	C			A	R	R	R					
8. Scale up JKN premium subsidy	A		R	I			I						R		
Carried: burning enforcement, fire water point	I	A				C	C	I							R

What the chart shows. **Dinkes Kota Ambon is accountable for three of the eight core actions and consulted on two more**, so it is the first meeting. **The mayor's office is accountable for the declaration and the insurance scale-up**, which unlock most of the public money. **Pemerintah Negeri Passo is the only body that is both Responsible and a funder at village level**, through its village fund.

WHERE MONEY DOES THE MOST

FUNDING

The Philanthropist lens, reconciled against the ranked actions and verified. Placements are ranked by marginal lives saved per dollar. The envelope is bounded by what this place can spend before the peak, not by what it needs. Nothing pre-arranged covers Ambon yet: IFRC's drought operation excludes Maluku, and the city's emergency reserve waits on a declaration.

FLOOR	TARGET	CEILING	LARGEST UNCERTAINTY
$30,000 (placements 1 and 3)	$55,500	~$107,000	Passo's population and its village-fund allocation; neither is published

* Suggested formulation: check with a health worker before sharing.

Target = $20,000 + $17,500 + $10,000 + $8,000. Ceiling = the average village-fund allocation per negeri in Kota Ambon (~$69,000: Rp33.81 bn ÷ 30 villages in 2025, our division), the largest channel shown to spend, plus $20,000 + $10,000 + $8,000. **The target fell from $88,700 (28 September) and from the lens's $73,000**, because a $35,000 match would have taken about half of Passo's likely village-fund allocation, and village funds are transfers, not a matching instrument.

RECIPIENT	USE	AMOUNT	MECHANISM	BASIS	VERDICT
Puskesmas Passo, via Dinkes Kota Ambon	Action 4: filters or chlorine, covered containers, ORS for under-5 households (net of Waimahu and Taholah, now on public water points)	$20,000	In-kind procurement, reported through health operational funds (BOK)	Filters RR 0.48 for diarrhoea (Clasen 2015); amount set by what the channel can absorb, not a unit cost	Partially confirmed; no record of spending at this scale
Pemerintah Negeri Passo with jumantik	Action 5: fitted lids, brushes, weekly checks	$17,500	Restricted grant alongside the negeri's own village-fund line	Camino Verde $7.47–16.72 per person-year (Nicaragua–Mexico); covers roughly 2,300–4,700 people for five months at those rates	Corrected (was $35,000 as a 1:1 match: contraindicated as sized)
BPBD / Pemkot Ambon	Action 3: tanker operating costs for Passo	$10,000	Standby reserve, released on the declaration	≈ Rp163m ÷ Rp250,000 per load ≈ 652 loads (price unverified)	Partially confirmed; fund operations, not trucks
Posyandu lansia (elderly health post); GPM diakonia possible co-deliverer	Action 7: weekly checks on isolated elders	$8,000	Direct grant	No cost-effectiveness study found	Unverified

The funder's one thing: the bottleneck is the channel, not the money. Fund only what a body has shown it can spend in five months, and the cheapest move is not a grant: it is getting Maluku into the IFRC operation (action 6) and the mayor's signature on the declaration (action 2). BAZNAS Kota Ambon paid out only Rp244.1m in all of 2024 across every programme, so it can't carry more than a small ORS request.

* Suggested formulation: check with a health worker before sharing.

DO NOT DO THIS CONTRAINDICATED

Advice that circulates for this hazard and would cost lives here.

Fogging or temephos (abate) as the main dengue response

CONTRAINDICATED

Pyrethroid resistance is pervasive in Indonesian *Aedes aegypti* (Ridha et al. 2025), and temephos resistance or tolerance is documented in Surabaya, Padang, Pekanbaru, Kuningan, Toraja and Central Java (Kasman et al. 2025). Nobody has tested Maluku. In Camino Verde, temephos added no protection.

Instead: fitted lids, weekly scrubbing, supervised checks (action 5); fog only around confirmed case clusters, and buy larvicide only after a local bioassay.

Handing out drums or improvised lids, or building open rainwater tanks this season

CONTRAINDICATED

Badly fitting lids still breed mosquitoes (14.8% positive in Yangon), and an open tank built now collects almost nothing before the rains return in April–May while becoming a breeding site.

Instead: manufactured fitted lids or tap-fitted tanks, with a brush and a check schedule.

Assuming tanker water is safe *

CONTRAINDICATED

Tankers are often filled from wells, and urban self-supplied groundwater in Indonesia is frequently contaminated (21.2% of 619 households above 100 E. coli per 100 mL in Hansen 2026; not Ambon data).

* Suggested formulation: check with a health worker before sharing.

Instead: test the fill source, chlorinate at the point of use, and pair every delivery with ORS for households with young children.

Standing the response down when the rains return

CONTRAINDICATED

The 2016 deaths came after the drought, and drought's effect on dengue recurs about two months later as storage habits change. Ambon's normal dengue season (June–November) follows the rains.

Instead: keep container checks and clinical readiness running through August 2027 at least.

A large 1:1 village-fund match for the container programme

CONTRAINDICATED AS SIZED

Village funds (Dana Desa) are national transfers, not matching money. A $35,000 match would take about half of Passo's likely annual allocation, 20% of which is already earmarked for food security, and would sit unspent past the dry season.

Instead: a $17,500 grant alongside whatever the negeri budgets through its own planning cycle.

WHAT HAS HAPPENED HERE BEFORE — THE RECORD

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
1997 (El Niño, ONI +2.4)	Aug–Oct rainfall 3, 1 and 7 mm against normals of 347, 231 and 132 mm	Not recorded	Not documented
1999–2004	Maluku conflict	Thousands across Maluku	Christian and Muslim settlement separated; pela alliances became bridges

* Suggested formulation: check with a health worker before sharing.

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
2016 (after the 2015 El Niño)	Dengue, Jan–Apr	37 cases, 4 deaths in Ambon; 2 deaths in Passo	Health office blamed uncovered water containers
2021	Ombudsman review of Ambon's water	Supply ~30% of demand, down up to 60% in dry season (not re-checked)	Catchment loss to housing named; not reversed
2023 (El Niño, ONI +2.0)	Dry-season water rationing	Batu Merah customers supplied once a week	Deep wells added elsewhere in the city
Jul–Aug 2025	Public water points opened in Waimahu and Taholah, Passo	After 19–26 years without	First public supply to those hamlets
Dec 2025 – Jan 2026	Tanker capacity added	—	BNPB unit to Ambon; four tankers to Batu Merah
Aug – Oct 2026	Burning appeal (15 Aug); Tual declares drought emergency (Sep); BMKG puts Ambon at *Waspada* (Oct)	—	Ambon has not declared

The recurrence nobody has noticed: Ambon's El Niño emergency starts when the rest of Indonesia's ends. The national rains arrive in November, just as Ambon enters its own dry season, and dengue follows in the new year and again through the June–November season. A response sized to the national calendar stands down just before Passo's deadliest months.

WHERE THE EXPERTS DISAGREE — UNRESOLVED

- **Use abate larvicide on stored water** (Emergency manager) versus **larvicide resistance is widespread in Indonesia and untested in Maluku** (Skeptic). Resolved as a gate: no larvicide purchase until a WHO bioassay on Ambon larvae.
- **Vector control first** (28 September ranking) versus **case management first, because Maluku's problem is case fatality, not case numbers** (Public health). This run ranks case management first; both stay in the plan.

* Suggested formulation: check with a health worker before sharing.

- **Dengue risk window January–April** (Historian, 2016) versus **Ambon's usual peak June–November** (Sasmono 2026). Both are real; readiness now runs November 2026 to November 2027.

WHAT NOBODY COVERED — BLIND SPOT

No lens could see **inside Puskesmas Passo or Haulussy**: beds, IV-fluid stock, staff trained in dengue management, or how long a referral actually takes. If case management is the lever, that capacity is the binding fact, and Haulussy is under external management support. Nor does anyone know **Passo's population or its village-fund allocation**, which set the size of every placement. And no source gave **2026 dengue or diarrhoea counts** for Ambon. One visit to the Puskesmas and one call to Dinkes would answer all three.

HOW TO READ THIS — METHOD

The panel is author-built. Seven simulated expert lenses were used to generate these recommendations. Where they agree, that is a strong hypothesis, not a consensus of the field. Every recommendation was then independently checked for evidence, transferability to this location, and feasibility before the peak; the verdict tags carry the result.

Mortality figures are resolved at Climate Impact Lab impact-region scale, roughly 300,000 people. They are not neighbourhood figures. The El Niño projection covers *heat only*; drought, flood, fire, smoke, and crop failure are additional to it and are carried here by the historical record rather than by a published number. Heat itself is modest here (monthly means 25–27 °C); fans are fine below 35 °C (Meade et al. 2024).

HOW THIS BRIEFING WAS PRODUCED — METHODOLOGY

- **Scope.** The location was taken as given: Negeri Passo, a customary village in Kecamatan Baguala (as Kemendagri and the village name it; Pemkot and BPS use Teluk Ambon Baguala), Kota Ambon, Maluku. This is a re-run of the 28 September 2026 report, requested by Angus with a RACI chart added. Climate Impact Lab impact region, as the report records it: not resolved. Mortality figures exist only at that scale (impact regions average roughly 300,000 people); sub-city detail is used for hazard exposure and action design, not for death counts.
- **Exposure.** Climate Impact Lab, *451,000 at Risk* (September 2026, pre-peer-review): ECMWF SEAS5 seasonal forecasts (51-member ensemble) applied to temperature–mortality relationships across 24,378 regions, against a 1996–2025 baseline, for June 2026 – February 2027. It covers heat only. It is never added to the Impact Map's long-run projections or quoted from the map's GDP-denominated mortality. Where heat is not the dominant hazard, the historical record carries the hazard instead of a borrowed number.

* Suggested formulation: check with a health worker before sharing.

- **Expert panel.** Seven simulated expert lenses (emergency manager, public health scientist, frontline responder, skeptic, economist, historian, and philanthropist) each researched this place independently and in parallel, from web sources, to a fixed brief. Each was given the 28 September findings to check and update rather than repeat.
- **Reconciliation.** Duplicate actions were merged and backing lenses recorded. Convergence was tested for shared bias (whether the evidence behind an agreed action came from a setting like this one) before it counted in an action's favour. Contradictions were kept, not resolved by dropping a side. Actions were ranked by expected lives saved per unit of effort before the peak, each was given a named payer and trigger or marked unfunded, and the funding placements were reconciled against the ranked list. The RACI chart was built from the owners named in the verified actions.
- **Verification.** Five verifier agents checked 13 of the 16 recommendations on three tests: evidence (the primary source and its effect size), transferability (whether that evidence came from somewhere like here), and feasibility (whether it can be done here before the peak). Financial actions were also checked for availability in this jurisdiction, a named payer, a trigger threshold, and whether they already exist. The funding envelope was checked for real recipients, published unit costs, and duplication, then re-summed. Three items carried from 28 September were not re-checked.
- **Verdicts.** *Confirmed*: evidence, transferability, and feasibility all hold. *Partially confirmed*: holds after the corrections shown. *Unverified*: could not be checked to a primary source. *Contraindicated*: would do harm or waste the window here.
- **Adaptation Inventory.** Not consulted this run: web search was exhausted, and its download sits behind a sign-up form. The 28 September run found no dengue entries for Indonesia.
- **Health wording.** Passages where a wording error could hurt someone (dengue danger signs and when to seek care, water treatment, and oral rehydration salts) are marked "* Suggested formulation: check with a health worker before sharing." They were checked against published sources, not by a health professional. They are suggestions, not medical advice, and anyone passing them on should confirm them with a doctor, pharmacist, nurse, or community health worker first. The marker was added on 2 November 2026.
- **Rules.** No cost is derived by dividing a published total, and no local figure is invented; where data is thin, the report says so and shows the containing geography's number with its arithmetic. Every source is listed below with its verification status.
- **Limits.** The panel and verifiers are AI research agents working from web-available sources, not human experts, and no one in this place was interviewed. Run-specific gaps: web search unavailable (agents fetched pages directly); BPS, BNPB, Kompas, ReliefWeb and the BPK regulations database returned 403; Passo's population, its village-fund allocation, Puskesmas Passo's capacity, 2026 dengue counts, tanker prices and the BTT figure were not verified; the "17 dengue cases in 2023" figure was dropped as unsourced.

* Suggested formulation: check with a health worker before sharing.

References · Dengue: Sasmono et al. 2026, Djaafara et al. 2026, Ambon dengue 2016, Ith et al. 2025, case-management training, Camino Verde (Andersson et al., BMJ 2015), Tschampl et al. 2020, Yangon lids, Malang jumantik, Kasman et al. 2025 (confirmed, corrected) · Water and health: Clasen et al. 2015 (Cochrane CD004794.pub3), Munos et al. 2010, GiveWell water quality, Siramaneerat 2026, Hansen 2026 (confirmed, corrected) · Drought and declaration: BMKG drought early warning, Tual declaration, Pemkot–BWS coordination, Waimahu and Taholah water points, Passo village site (confirmed) · Finance: IFRC MDRID030 document, IFRC trigger report, JKN Rp8.9 bn, Titaley et al. 2025, Dana Desa ranking, BAZNAS 2024 (confirmed, corrected) · Place: Teluk Ambon Baguala, pela makan patita 2024 (confirmed) · Seasons: UNPATTI rainfall 1959–2018; ENSO: NOAA ONI · Heat: Meade et al. 2024

Exposure data · Climate Impact Lab, 451,000 at Risk · Impact Map · Adaptation Inventory

Generated 2 October 2026 for Passo, Baguala, Kota Ambon, Indonesia (re-run of 28 September). El Niño Ready is volunteer-run and is not affiliated with Climate Week NYC, The Climate Group, or Columbia University.

* Suggested formulation: check with a health worker before sharing.